4	Perform hand hygiene and put on disposable gloves if available	To prevent the spread of infection
5	Position the client with the head of the bed elevated at least 30 degree angle to 45 degree angle	This position helps avoiding aspiration of feeding solution into lungs
6	Determine placement of feeding tube by: Aspiration of stomach secretions Attach the syringe to the end of feeding tube Gently pull back on plunger Measure amount of residual fluid Return residual fluid to stomach via tube and proceed to feeding Nursing Alert If amount of the residual exceed hospital protocol or Dr.'s order, refer to these order	Aspiration of gastric fluid indicates that the tube is correctly placed in the stomach The amount of residual reflects gastric emptying time and indicates whether the feeding should continue. Residual contents are returned to the stomach because they contain valuable electrolytes and digestive enzymes. In the case of non present of residual, you should check placement carefully. Residual over 120 mL may be caused by feeding too fast or taking time more to digest. Hold feeding for 2 hours, and recheck residual.
	Injecting 10 – 20 ml of air into tube Attach syringe filled with air to tube Inject air while listening with stethoscope over left upper quadrant	Inject 3-5mLof air for children A whooshing or gurgling sound usually indicates that the tube is in the stomach
7	Taking an x-ray or ultrasound	It may be needed to determine the tube's Placement
	Intermittent or Bolus feeding Using a feeding bag: Feeding the following 1) Hang the feeding bag set-up 12 to 18 inches above the stomach. Clamp the tubing. 2) Fill the bag with prescribed formula and prepare the tubing by opening the clamp. Allow the feeding to flow through the tubing Re clamp the tube. 3) Attach the end of the set-up to the gastric tube. Open the clamp and adjust flow according to the Doctor's order. 4) Add 30-60 ml of water to the feeding bag as feeding is completed. Allow the flowing to basin. 5) Clamp the tube and disconnect the feeding set-up.	Rapid feeding may cause nausea and abdominal cramping. Water clears the tube, keeping it patent. Clamping when feeding is completed prevents air from entering the stomach

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